AMRAKUNJ CANCER CARE
Radiation Oncology | Gynaec Oncology | Day-Care Chemotherapy
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Reg. No. GJ/AMD/HOSP/2016/0482

RADIATION ONCOLOGY OPD

First consultation

Pt.:	**HANSABEN JAYANTILAL PATEL**	Reg. No.:	**ACC/26/1874**
Age / Sex:	59 / F	Dt:	**08/04/26**
Consultant:	Dr. Nirali Desai	Ref. by:	Dr. Bhavna Mehta (Gynaec)

History

c/o post-menopausal PV bleeding x 4 months, foul-smelling discharge, low backache. k/c/o DM-2 x 8 yrs on OHA, HTN x 5 yrs. P3L3, all FTND. Menopause 9 yrs back.

Examination

ECOG 1. Pallor +. No pedal oedema; no palpable groin or supraclavicular nodes. P/S: exophytic growth replacing the cervix, bleeds on touch. P/V: growth about 5 cm, upper 1/3 of the vagina involved. P/R: both parametria involved, the right reaching the lateral pelvic wall.

Investigations

- HPE (cervical punch biopsy) dtd 28/03/2026: keratinising squamous cell carcinoma, moderately differentiated.
- MRI pelvis dtd 2.4.2026: 5.4 x 4.8 cm cervical mass with bilateral parametrial extension reaching the right pelvic side wall; mild right hydroureteronephrosis; no enlarged pelvic nodes.
- CBC and RFT (reports of 06/04/2026): Hb 9.6 g/dL, S. creatinine 1.0 mg/dL.

Imp.

Ca cervix (SCC), FIGO 2018 stage IIIB.

Plan

- Definitive concurrent chemoradiation: EBRT 50 Gy / 25 # (IMRT) with weekly cisplatin 40 mg/m2, followed by HDR intracavitary brachytherapy 7 Gy x 3 #.
- RT planning CT simulation on 13/04/2026; bladder and bowel protocol explained.
- Physician fitness for DM / HTN; dental check-up. Build up Hb (target above 10 g/dL).
- Explained to the patient and her son in Gujarati; consent for RT taken.

Dr. Nirali Desai
M.D. (Radiation Oncology)
Reg. No. G-31877
Consultant Radiation Oncologist

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